

ENGLISH FOR WORK / SEPTEMBER 2026

Medical Laboratory Technician English



Vocabulary & phrasebook

Keep precise meanings and useful expressions close. Retrieve the words, then use them in a complete message.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Medical laboratory technicians and clinical laboratory support staff.

All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Make vocabulary usable

Knowing a definition is a start. To use a term well, explain it in plain English, connect it to a case, and choose a natural sentence around it.

- Read five terms, cover their definitions, and recall the meanings.
- Sort a pair you might confuse and explain the difference.
- Use one term in a sentence about a fictional case.
- Ask a partner to explain the sentence without repeating the specialist word.
- Return to the same terms on another day and test recall again.

Abbreviations need context

Expand an abbreviation on first use when the listener needs it. Some abbreviations have different meanings across professions: API can refer to an application programming interface or an active pharmaceutical ingredient. The course context determines the meaning.

Definitions have limits

These are concise language-learning explanations, not complete legal, clinical, or technical specifications. Use the applicable authoritative definition for regulated or operational decisions.

Field vocabulary A-Z

acceptance criteria

The laboratory's defined requirements for determining whether a specimen is suitable for specified testing.

accession number

A laboratory-assigned identifier linking a received specimen or case to its testing records.

accessioning

The laboratory's process of registering a specimen or case and associating it with its order.

add-on request

A request to perform additional testing on an existing specimen, subject to order and suitability requirements.

amended report

A report revised after issue, with the change recorded through the laboratory's correction process.

analytical run

A defined group or sequence of laboratory testing performed under specified conditions.

backlog

Laboratory work waiting to be completed beyond the work currently being processed.

chain of custody

A documented record of possession and transfer used when required for a specimen's particular purpose.

control material

Material with assigned or expected characteristics used to monitor a test system's performance.

critical result

A result meeting the laboratory's established criteria for prompt notification through its designated clinical pathway.

downtime

A period when a laboratory instrument or service is unavailable for its intended work.

escalation pathway

The designated sequence of contacts and actions used when the required response or recipient is not reached.

final report

A report issued at the completed reporting stage for the specified request, subject to any later correction.

hemolysis

Breakdown of red blood cells that can affect some laboratory measurements, requiring method-specific assessment.

insufficient quantity

A specimen amount that does not meet the laboratory's requirements for the requested testing.

lot verification

The laboratory's checks establishing whether a particular material lot meets requirements for use.

maintenance record

Documentation of service, checks, and related actions for laboratory equipment.

notification log

The laboratory record of result-notification attempts, recipients, acknowledgments, and times.

preliminary report

A report issued before all relevant testing or review is complete, with its stated limitations.

quality control

Checks used to monitor whether a laboratory testing process is performing within its established requirements.

read-back

Repeating communicated details so the sender can check that they were received accurately.

reagent

A substance used in a laboratory test to produce or measure a reaction.

recollection

Obtaining another specimen when the laboratory or care team determines one is needed.

reference interval

A comparison range supplied with some test results, derived for a defined reference population and method.

referral laboratory

An external laboratory that performs testing sent from another laboratory or service.

release authorization

The required approval allowing laboratory results to be made available for reporting.

requisition

The clinician's paper or electronic request identifying the laboratory tests ordered.

send-out test

A test whose specimen or material is sent to another laboratory for performance.

shipping manifest

A document listing items included in a shipment.

specimen label

The identifying information attached to a specimen container under the laboratory's requirements.

STAT priority

A designation for urgent testing under the laboratory's established priority and response arrangements.

turnaround time

The interval between defined starting and ending events in a laboratory service, such as receipt and reporting.

Expressions for this course

Complete the frames with the facts of your case. A frame helps organize meaning; it should not replace an actual explanation.

Resolve the reference before answering the status question

Your enquiry says ____; the receiving log lists ____.

Could you read the complete ____ back?

We have not yet established that ____ refers to ____.

Repeat the complete identifier and name its source. A similar name, location, or code is not a verified match. These exchanges do not replace the laboratory's identity requirements.

Word combinations in this lesson

confirm the accession number: establish which laboratory case is being discussed

match the requisition to the label: compare the order and specimen identifiers

read back an identifier: repeat a complete code to check accurate communication

report an accessioning discrepancy: identify an unresolved registration mismatch

Distinguish a specimen problem from a patient result

____ describes the specimen, not a patient result.

The documented decision for request ____ is ____.

The care team needs to confirm ____.

Insufficient quantity concerns available sample, not a measured patient level. State the documented decision and its request reference. Do not generalize one specimen issue to every requested test.

Word combinations in this lesson

review specimen suitability: assess whether the specimen meets the relevant requirements

state the rejection reason: report the documented reason testing cannot proceed on that specimen

request recollection clarification: ask which specimen or request a recollection decision concerns

avoid assigning collection blame: describe a specimen issue without inventing who caused it

Resist an inaccurate final-status label

The run is complete; ___ remains unresolved.

Completion does not establish ___.

I can confirm ___, but cannot report ___ as final.

Describe the stage without teaching control interpretation or corrective testing. Unresolved review does not establish that every patient result is wrong. Keep the release decision with the laboratory's qualified reviewer.

Word combinations in this lesson

review control performance: assess the recorded quality-control information

hold results for review: keep results from release while a required issue is unresolved

confirm run status: establish whether a testing run has met its review requirements

separate completion from release: distinguish finished instrument work from permission to report

Ask what a service completion actually establishes

The service visit ended at ___; ___ remains unconfirmed.

Could you confirm whether ___ is available for ___?

Who owns the next update on ___?

A service engineer's completed visit and the laboratory's permission to resume testing are separate. State the instrument reference, observed status, and exact question. Do not guess settings, restart methods, or cause.

Word combinations in this lesson

identify the reagent lot: specify the exact material batch being discussed

check verification status: establish whether required material review is complete

log instrument downtime: record the period and scope of unavailability

coordinate a service enquiry: give equipment support the facts needed for review

Name the starting point before discussing the estimate

Our quoted interval runs from ___ to ___.

___ is the reference point, not ___.

The current estimate is affected by ___; ___ remains unconfirmed.

Turnaround intervals need defined start and end events. Correcting the starting point does not eliminate delays. STAT is a priority designation, not a promise that a particular result already exists.

Word combinations in this lesson

define the turnaround clock: state which events start and end the quoted interval

update the expected reporting time: revise a timing estimate using current information

clarify the stated priority: check the requested urgency against the recorded order

assess an add-on request: obtain review of whether extra testing can be accepted

Correct the report label and verify the version

I incorrectly called ___ final; it was ___.

The version issued at ___ supersedes ___.

Please confirm which report your team is viewing.

A final label describes a reporting stage, not an assurance that a patient is healthy or that no correction could ever follow. Use the reference, issue time, and version together.

Word combinations in this lesson

identify the report version: state which issued document is being discussed

explain preliminary status: distinguish an interim report from a completed reporting stage

notify a report amendment: tell the receiving team that an issued report has changed

refer interpretation questions: direct clinical meaning questions to the responsible clinician

Complete the read-back and correct what was misheard

For verified case ___: [analyte], [value], [units].

You repeated ___; the reported value is ___. Please read it back.

That read-back matches. Please confirm you accept this notification.

The sender states the information, the receiver repeats it, and the sender confirms or corrects it. Analyte means the substance measured. Millimoles per liter (mmol/L) and milligrams per deciliter (mg/dL) are different units; do not interchange or convert them in these exercises. All critical designations are fictional lab-specific facts, not universal thresholds.

Word combinations in this lesson

verify the receiving clinician: establish that the intended qualified recipient is on the line

obtain a read-back: request repetition of the critical communicated details

record acknowledgment: document that the intended recipient confirmed receipt

escalate an unanswered notification: use the designated pathway when the required contact is not achieved

Transfer the item-level question, not just the parcel status

The manifest lists __; the receiving laboratory has confirmed __.

The outstanding question is __ for accession __.

Please confirm you accept both __ and __.

A manifest lists intended contents; parcel delivery does not itself confirm item acceptance. Chain-of-custody requirements apply only in their proper context, not automatically to every routine specimen.

Word combinations in this lesson

reconcile the shipping manifest: compare the shipment list with confirmed received items

confirm referral-laboratory acceptance: check whether the external laboratory accepted the request

handover a send-out enquiry: transfer the unresolved external-testing question to a named colleague

trace a shipment discrepancy: establish which transfer or receipt detail remains unknown

Model responses in context

Read the situation first. Cover the model, say your own response, and compare the two for meaning and tone.

01 | Accessioning and unambiguous identifiers

The receiving log lists accession A314; a clinic caller asks about A341. No identity match has been established. Technician Noor can check the accession log. The caller must identify the requested case through the laboratory's privacy and verification arrangements.

Before we discuss a specimen status, could we clarify the accession number? Our receiving log lists A314, while your enquiry says A341. Those are different identifiers. Please check the reference on your request through our verification arrangements. I will keep the enquiries separate until we establish which case you mean; no match has been confirmed yet.

Try it again: Say both cases came from the same clinic and ask whether that is enough to match them.

AI extension: build and retrieve vocabulary from this case.

02 | Specimen suitability and recollection language

The laboratory lead has documented insufficient quantity for test request Q6 and requested recollection through the clinic. No collection method, urgency, or cause is supplied. The clinic thinks insufficient quantity means a low patient result.

For Q6, insufficient quantity describes the amount of specimen available, not a low result in the patient's blood. The laboratory lead has requested recollection through your clinic. There is no reportable result from this request in this case. Please confirm that the care team has received the recollection message; the collection arrangements belong with them.

Try it again: Ask whether the collector definitely caused the problem; the cause is not established.

AI extension: build and retrieve vocabulary from this case.

03 | Quality-control status and reportable results

An analytical run is complete, but its quality-control review is unresolved. The laboratory lead has not authorized result release. A clinic wants results described as final because the instrument finished. No values or troubleshooting instructions are supplied.

The analytical run has finished, but the quality-control review remains unresolved and release has not been authorized. I cannot describe these results as final just because the instrument completed its work. The laboratory lead is reviewing the issue. I can report that status to your team, but I do not have a confirmed release time.

Try it again: Ask whether the quality review proves the patient results are inaccurate.

AI extension: build and retrieve vocabulary from this case.

04 | Reagents, instruments, and service coordination

Analyzer H2 is marked unavailable. A service visit ended at 11:00, but the laboratory lead has not confirmed return to service. Technician Noor needs a status from the lead before updating the clinic. No repair steps, settings, or patient results are supplied.

Could you confirm H2's return-to-service status? The service visit ended at eleven, but I do not see the laboratory lead's confirmation that testing can resume. I need to update the clinic accurately. Please distinguish the completed service visit from any checks or release decision still outstanding, and tell me who owns the next status update.

Try it again: Ask whether the instrument's illuminated screen proves it is available for testing.

AI extension: build and retrieve vocabulary from this case.

05 | Turnaround times and realistic updates

The service's quoted two-hour turnaround is measured from laboratory receipt to reporting. Specimen T5 was collected at 08:00 and received at 09:00. A backlog affects estimates; no report time is confirmed. The clinic assumes collection started the two-hour clock.

The quoted two-hour turnaround is measured from laboratory receipt, not collection. T5 was received at nine after collection at eight. That distinction changes the reference point, but the current backlog means I still cannot promise a report at eleven. The section lead can provide the latest estimate; please treat it as an estimate unless confirmed otherwise.

Try it again: Ask whether adding STAT to your telephone message independently changes the recorded order priority.

AI extension: build and retrieve vocabulary from this case.

06 | Preliminary, final, and amended reports

The clinic received a preliminary report at 10:00 and a final report at 14:00. A technician called the earlier version final by mistake. The final report supersedes the preliminary version for this request. No findings or clinical conclusions are supplied.

I need to correct my earlier description. The ten o'clock report was preliminary, not final. The final report was issued at two and supersedes that earlier version for this request. Please confirm that your team is viewing the two o'clock report. I can clarify the version and status, but clinical interpretation belongs with the responsible clinician.

Try it again: Ask whether a value inside a reference interval settles the whole clinical question.

AI extension: build and retrieve vocabulary from this case.

07 | Critical notifications and acknowledged handoffs

Fictional laboratory Lumen designates C7's potassium result of 6.4 mmol/L critical under its own criteria; mmol/L means millimoles per liter. Case identity and Dr Shah's receiving authority are already verified. The report was issued at 14:10; this call occurs at 14:12. Dr Shah's contact is extension 214. Dr Shah initially repeats 6.1. Correct the value, obtain the complete read-back, and confirm receipt. No diagnosis, treatment, or universal threshold is supplied.

Dr Shah, Noor from Lumen laboratory. For C7, the critical potassium result is six point four millimoles per liter. You repeated six point one; it is six point four. Please read back the complete result. That matches, thank you. The report was issued at two ten; I'll record your receipt at two twelve and contact extension 214.

Try it again: Ask whether six point four proves a diagnosis. The technician preserves the reported value and leaves clinical interpretation to the clinician.

AI extension: build and retrieve vocabulary from this case.

08 | Referral laboratories and outstanding work

Send-out X8 appears on Monday's manifest. The courier confirms parcel delivery Tuesday, but the referral laboratory has not confirmed X8's accession acceptance. Noor's shift ends; incoming technician Eli accepts the enquiry and an agreed 15:00 clinic update. No result date is confirmed.

Eli, X8 is listed on Monday's manifest, and parcel delivery was confirmed Tuesday. We still lack the referral laboratory's accession acceptance for X8, so a result date is not established. Please take the acceptance enquiry and the clinic update agreed for three. Confirm that you have both tasks; parcel delivery alone does not close this send-out follow-up.

Try it again: Ask whether a courier signature counts as referral-laboratory accession acceptance.

AI extension: build and retrieve vocabulary from this case.

Use the language responsibly

All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

U.S. Bureau of Labor Statistics: Clinical Laboratory Technologists and Technicians

<https://www.bls.gov/ooh/healthcare/clinical-laboratory-technologists-and-technicians.htm>

Centers for Disease Control and Prevention: About Laboratory Quality

<https://www.cdc.gov/lab-quality/php/about/>

Agency for Healthcare Research and Quality: TeamSTEPPS Check-Back (or Repeat-Back)

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/checkback.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Guided message practice gives four choices with explained feedback. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-28. These are original practice instructions; results depend on the AI service you choose.

Build vocabulary and collocations

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Medical Laboratory Technician English

Professional audience: Medical laboratory technicians and clinical laboratory support staff

Scope: All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

Lesson 01: Accessioning and unambiguous identifiers

Original fictional case: The receiving log lists accession A314; a clinic caller asks about A341. No identity match has been established. Technician Noor can check the accession log. The caller must identify the requested case through the laboratory's privacy and verification arrangements.

Language workshop: Resolve the reference before answering the status question

Communication goal: Clarify similar laboratory identifiers without merging cases.

Language guidance: Repeat the complete identifier and name its source. A similar name, location, or code is not a verified match. These exchanges do not replace the laboratory's identity requirements.

Useful frames (complete the gaps with case facts): Your enquiry says ____; the receiving log lists _____. / Could you read the complete _____ back? / We have not yet established that _____ refers to _____.

Vocabulary: accession number: A laboratory-assigned identifier linking a received specimen or case to its testing records. / requisition: The clinician's paper or electronic request identifying the laboratory tests ordered. / specimen label: The identifying information attached to a specimen container under the laboratory's requirements. / accessioning: The laboratory's process of registering a specimen or case and

associating it with its order.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic caller: your note says A341; check the actual request when asked.

Guided response task: Complete a workplace dialogue by selecting one of four supplied options. Compare the feedback and say the completed exchange aloud. Do not write an open-ended message.

Optional second-round challenge: Say both cases came from the same clinic and ask whether that is enough to match them.

Model for comparison AFTER my attempt, not a response to give me first: Before we discuss a specimen status, could we clarify the accession number? Our receiving log lists A314, while your enquiry says A341. Those are different identifiers. Please check the reference on your request through our verification arrangements. I will keep the enquiries separate until we establish which case you mean; no match has been confirmed yet.

END REFERENCE

TASK: Retrieve occupational vocabulary

1. Select four terms from the reference. Explain each in plain English, give two natural collocations and quote or create a short workplace example. Label invented examples. Contrast each term with a plausible confusable term; acknowledge regional variation.
2. Present one role-labelled exchange with one missing word or phrase and four choices A-D. Exactly one choice must fit the stated facts and grammar. Stop and wait for my answer before revealing it.
3. Explain all four options, then invite me to read the completed exchange aloud. Offer a sentence frame for optional spoken practice, never an open-ended writing assignment.
4. Repeat for the other three terms, one question at a time. Vary the correct letter and the situation; preserve the reference facts. If a choice is ambiguous, replace the item.
5. Finish with two new four-choice collocation questions for later review, with a clearly separate answer key. Do not claim to schedule a reminder.

END PROMPT

Test recall and transfer

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Medical Laboratory Technician English

Professional audience: Medical laboratory technicians and clinical laboratory support staff

Scope: All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

Lesson 01: Accessioning and unambiguous identifiers

Original fictional case: The receiving log lists accession A314; a clinic caller asks about A341. No identity match has been established. Technician Noor can check the accession log. The caller must identify the requested case through the laboratory's privacy and verification arrangements.

Language workshop: Resolve the reference before answering the status question

Communication goal: Clarify similar laboratory identifiers without merging cases.

Language guidance: Repeat the complete identifier and name its source. A similar name, location, or code is not a verified match. These exchanges do not replace the laboratory's identity requirements.

Useful frames (complete the gaps with case facts): Your enquiry says ____; the receiving log lists _____. / Could you read the complete _____ back? / We have not yet established that _____ refers to _____.

Vocabulary: accession number: A laboratory-assigned identifier linking a received specimen or case to its testing records. / requisition: The clinician's paper or electronic request identifying the laboratory tests ordered. / specimen label: The identifying information attached to a specimen container under the laboratory's requirements. / accessioning: The laboratory's process of registering a specimen or case and

associating it with its order.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic caller: your note says A341; check the actual request when asked.

Guided response task: Complete a workplace dialogue by selecting one of four supplied options. Compare the feedback and say the completed exchange aloud. Do not write an open-ended message.

Optional second-round challenge: Say both cases came from the same clinic and ask whether that is enough to match them.

Model for comparison AFTER my attempt, not a response to give me first: Before we discuss a specimen status, could we clarify the accession number? Our receiving log lists A314, while your enquiry says A341. Those are different identifiers. Please check the reference on your request through our verification arrangements. I will keep the enquiries separate until we establish which case you mean; no match has been confirmed yet.

END REFERENCE

TASK: Review with supported choices and speaking

1. Ask me to close my notes. Prepare six four-choice questions: two vocabulary gaps, one grammar contrast, one confirmed-versus-unknown distinction, one choice of reply, and one transfer to a clearly labelled fictional variation.
2. Show only the first question. Use a short role-labelled exchange or a concrete scenario, four plausible choices A-D, and exactly one defensible answer. Stop and wait.
3. After each answer explain all options, offer one hint for a retry if needed, and invite me to say the completed exchange aloud. Only then move on. Do not request an original written response.
4. Track whether I answered independently or with a hint, not a certified proficiency score. Do not assess pronunciation from typed text. Replace ambiguous questions and accept valid spoken alternatives.
5. Give a compact review card with three four-choice recall items and a separate answer section. Summarize two specific successes and two points to revisit. Do not claim to remember or schedule future sessions.

END PROMPT